

SEGMENT SLICK · RHEUMATOLOGY

The Rheumatology Play

Field-tested by Paul. Polished up for the rest of you. Use it, steal it, beat it.

Why Rheumatology, Why Now

Here's the unlock: in 2023, the ACR and CHEST jointly published the first clinical guideline for screening and monitoring ILD in patients with systemic autoimmune rheumatic diseases. The guideline names **PFTs — spirometry, lung volumes, AND DLCO — as a recommended diagnostic test** for ILD in RA, scleroderma, Sjögren's, lupus, and inflammatory myopathies.

That's the door. Here's the patient math behind it:

50%+

Of scleroderma patients develop ILD

10–30%

Of RA patients develop ILD

DLCO

Most sensitive early marker for ILD in connective tissue disease — the MiniBox+ runs it as part of a complete PFT, not a bolt-on

For a rheumatologist with high RA and SSc volume, the PFT population is already in the chart. They're just sending it out the door.

What Dr. Torres Looked Like on Paper

Two locations. **About 20 PFTs a week per location**, all being referred out to imaging or hospital labs. RA-heavy practice. Active in DMARD and biologic management — which means recurring PFTs, not one-and-done.

When the volume is already there, the clinical argument writes itself.

The Four Questions That Got Paul to the Demo

Paul didn't pitch on the discovery call. He asked. These are the four. Use them.

1 "What percentage of your patients are on DMARDs or biologics, and how do you currently monitor them for pulmonary toxicity?"

2 "For your RA and scleroderma patients, where are you sending PFTs today — and how long until you get the results back?"

3 "When a patient's DLCO trends down, what does your workflow look like to catch it?"

4 "How would your treatment cadence change if you owned the PFT data inside your practice?"

The fourth one is the closer. Once a rheumatologist pictures trended DLCO data in their own chart, the math on the conversation shifts.

How to Use This

Paul's path — discovery call, guideline-anchored email, demo, financing conversation — is the pattern. The questions, the objections, the CPT bundle, and the financing math travel from territory to territory.

One ask: if you call on a rheumatology practice using this framing, send the outcome back. We're building this segment one named practice at a time.

Why DMARDs and Biologics Are Your Friend in This Conversation

This is the second proof point most reps will miss. Both drug classes make the PFT case stronger — and you should know enough to talk about them.

DMARDs

Methotrexate, hydroxychloroquine, sulfasalazine. The backbone of RA treatment. They slow disease progression by modulating the immune system. Patients on them get monitored long-term.

Biologics

Humira (adalimumab), Enbrel (etanercept), Rituxan (rituximab), Orencia (abatacept). Targeted DMARDs that block specific inflammatory proteins. Also long-term. Also monitored.

Here's the hook: several of these drugs carry a risk of pulmonary toxicity as a side effect. A rheumatologist managing a patient on methotrexate or a TNF inhibitor isn't just watching for RA-ILD progression. They're watching for drug-induced lung injury. Repeat PFTs are standard of care for both.

For Dr. Torres, that's two clinical reasons to test the same patient. Recurring testing. Recurring data. Both inside the practice.

The Objections Paul Handled. The Answers You Should Memorize.

OBJECTION	HOW PAUL HANDLED IT
"My staff isn't trained on PFTs."	No RT required. We train the MA, RN, or NP. Six buttons, start to finish.
"Will I get reimbursed? I'm a rheumatologist, not a pulmonologist."	Yes. The 2023 ACR/CHEST guideline gives you clean medical-necessity documentation. Document the underlying diagnosis and symptoms (dyspnea, cough, crackles) on the order. The standard MiniBox+ bundle for ILD screening is 94060 + 94726 + 94729 .
"\$15K upfront is a stretch."	Dr. Torres went lease-to-own. Zero upfront. 60-month term, first three months at \$0, then 57 payments of \$793.94. \$1 buy-out at the end.

The CPT Bundle. Memorize It.

For ILD screening in autoimmune rheumatic patients, the standard MiniBox+ bundle is:

94060	Pre vs. post bronchodilator spirometry
94726	Plethysmography for lung volumes (TLC)
94729	Diffusing capacity (DLCO)

Document symptoms (dyspnea, cough, crackles on auscultation) or the underlying diagnosis on the order. Medical necessity lives in the documentation, not the code.

THE HEADLINE FOR THIS SEGMENT

Their patients need PFTs. The guideline says so. Now they can run them.

A complete PFT for the rheumatology practice. Spirometry, lung volumes, DLCO. In office. In 15 minutes.